

JOHNS HOPKINS MEDICINE – NEUROLOGY REFERRAL INTAKE (FAXED)

Received: 03/11/2026 14:22 EST

Fax ID: 443-955-xxxx

Referring Office: NOVA Family Practice (unclear stamp)

Referral Reason (as written): “abnormal movements ?seizure vs tremor vs other”

Priority: Routine (but patient requesting sooner appt per note)

Requested Service: Neurology (Movement? Epilepsy? TBD)

Insurance: BCBS PPO (card not attached)

Referring Provider: A. Mehta, MD (signature partially cut off)

Contact: 703-555-8123

Comments:

“Pt with episodic shaking, unclear etiology, prior ED visit. MRI ok. EEG ?done. Please eval and treat. Patient anxious re: diagnosis.”

Handwritten addendum:

“family worried about Parkinsons?? also spells of unresponsiveness???”

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PATIENT DEMOGRAPHICS (PRINTED FROM EHR)

Name: Carter, Melissa J

DOB: 02/18/1991

Age: 34

Sex: F

Address: 14722 Bramblewood Ct, Centreville, VA 20120

Phone: 571-555-2289 (mobile), 703-555-1182 (home?)

Email: mjcarter91@gmail (cut off)

Emergency Contact: Daniel Carter (spouse) 571-555-3321

Primary Insurance: BCBS PPO

Secondary: none listed

Employer: “self-employed / Etsy store”

Preferred Language: English

Interpreter: No

Allergies:

NKDA (but elsewhere “penicillin rash as child?”)

Height: 5’6”

Weight: 172 lb (recent), prior note 158 lb

Smoking: former smoker (quit 2019)  
Alcohol: occasional wine (2-3/wk) vs “denies” in older note  
Illicit: denies

Past Medical History (auto-populated):  
Anxiety disorder  
Migraine headaches  
IBS  
“possible seizure disorder?” (entered 01/2026)  
GERD  
Seasonal allergies

Past Surgical History:  
Appendectomy (2008)  
“wisdom teeth extraction”

Family History:  
Mother: hypothyroid, “tremor?” unclear  
Father: HTN  
No known epilepsy per pt, but one note says “cousin with seizures”

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REFERRING PROVIDER NOTE  
Date: 02/28/2026  
Provider: A. Mehta, MD

Chief Complaint: abnormal movements

HPI:  
34F with hx anxiety, migraines presenting for evaluation of ongoing abnormal movements x approx 5-6 months. Patient reports episodes of shaking involving arms and occasionally whole body, sometimes with “spacing out.” Frequency variable, up to several times daily. Duration seconds to minutes. No clear triggers though worse with stress.

She has been seen in ED twice (Dec + Jan). MRI reportedly normal. EEG ordered but unclear if completed (patient says yes, records not available).

Movements described as “jerky, sometimes rhythmic,” occasionally suppressible per patient (but spouse says not). No tongue biting or incontinence reported. Post-episode fatigue sometimes.

Denies focal weakness but notes intermittent numbness L hand.

Also reports tremor at rest at times (not observed today).

Exam today: inconsistent findings, intermittent tremor R hand when distracted? Gait normal. No rigidity.

Assessment:

Abnormal movements, unclear etiology. Differential includes seizure disorder, functional etiology, movement disorder, less likely metabolic.

Plan:

Refer to Neurology (Hopkins per patient request)

Continue sertraline

Trial propranolol PRN (for tremor/anxiety)

Follow up after neurology

Additional note:

Patient very concerned about serious neurologic disease despite reassurance.

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PRIOR NOTE #1 (ED VISIT)

Facility: Inova Fair Oaks Hospital

Date: 12/19/2025

Time: 22:14

Chief Complaint: "seizure?"

HPI:

Pt is 34F brought in by husband after episode of shaking while watching TV. Husband reports full body shaking lasting ~2 min. Eyes closed. No cyanosis. No incontinence. Pt recalls hearing voices during event but unable to respond.

No prior hx seizures per pt (though chart shows "possible seizure disorder?" entered later).

ROS: negative except anxiety, headache earlier in day.

Exam:

Vitals stable.

Neuro: alert, oriented. No focal deficits.

Observed episode in ED waiting area described by nurse as "irregular shaking of arms, stops when staff enters room."

Labs:

CBC, CMP WNL

EtOH negative

Imaging:

CT head w/o contrast: no acute intracranial abnormality

Assessment:  
Possible seizure vs psychogenic non-epileptic event.

Disposition:  
Discharged home  
Follow up neurology  
Driving precautions discussed (unclear documentation)

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PRIOR NOTE #2 (PRIMARY CARE FOLLOW-UP)  
Date: 01/05/2026  
Provider: NP L. Simmons

CC: follow-up ED visit

HPI:  
Pt reports continued episodes of shaking, now daily. Describes episodes as “tremor starting in hands then spreading.” Denies loss of consciousness (contradicts ED note). Reports she can sometimes “stop it if I focus.”

Also endorses significant stress, poor sleep.

Review of ED records:  
CT negative. No EEG done.

Exam:  
General anxious appearing  
Neuro:  
tremor noted intermittently, variable amplitude, distractible  
strength 5/5  
reflexes normal

Assessment:  
Tremor vs anxiety-related symptoms vs seizure.

Plan:  
Start sertraline 25 mg daily  
Refer neurology  
Labs ordered (TSH, B12)

Copy-forward (from previous visit unrelated):  
Patient also here for IBS management, continues dicyclomine PRN.

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PRIOR NOTE #3 (NEUROLOGY CONSULT – COMMUNITY)

Provider: Dr. R. Khanna

Date: 01/22/2026

Reason: abnormal movements

HPI:

34 yo female with 6 mo hx episodic shaking. Episodes variable in description. Sometimes unilateral arm shaking, sometimes generalized. Pt reports occasional “blinking out,” though remains aware per history.

No tongue biting, no incontinence. No clear post-ictal state.

Past hx migraines, anxiety.

Medications:

sertraline, sumatriptan, omeprazole, loratadine

Exam:

Mental status normal

Cranial nerves intact

Motor: full strength

Tone normal

Observed tremor during exam that changed frequency with distraction

Gait normal

Impression:

Events not clearly epileptic. Consider functional etiology vs less likely focal seizures.

Plan:

Order EEG

MRI brain (if not already done)

Follow up

Addendum:

Patient requested referral to tertiary center (Hopkins).

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LAB RESULTS (VARIOUS DATES)

01/06/2026

TSH: 1.8 (normal)

B12: 412 (normal)

12/19/2025 (ED)

CBC:

WBC 6.2

Hgb 13.4

Plt 244

CMP:

Na 139

K 4.1

Glucose 98

Cr 0.8

01/15/2026

ANA: negative

ESR: 8

Random labs (unclear relevance):

Vitamin D: 22 (low)

Lipid panel:

LDL 142 (high)

HDL 48

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## IMAGING REPORT

MRI Brain w/wo contrast

Date: 01/30/2026

Facility: Fairfax Imaging Center

Findings:

No acute infarct, hemorrhage, or mass.

No abnormal enhancement.

Ventricles normal in size.

No demyelinating lesions identified.

Impression:

Unremarkable MRI brain.

EEG REPORT (partial, fax cut)

Date: 02/03/2026

Routine EEG performed.

Background normal.

No epileptiform discharges seen.

No events captured.

Impression:  
Normal EEG.

(handwritten: "doesn't rule out seizures")

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#### CURRENT MEDICATION LIST (EHR AUTO-GENERATED)

Sertraline 50 mg daily  
Propranolol 10 mg PRN (started recently)  
Sumatriptan 50 mg PRN migraine  
Omeprazole 20 mg daily  
Loratadine 10 mg daily  
Dicyclomine 20 mg PRN  
Vitamin D 2000 IU daily  
Multivitamin  
Melatonin PRN sleep  
Ibuprofen PRN

Old meds still listed:  
Alprazolam 0.25 mg PRN (pt says not taking)  
Topiramate (stopped 2024?)

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#### DISCHARGE SUMMARY (ED VISIT #2)

Date: 01/14/2026

Reason for Visit:  
Recurrent shaking episodes

Course:  
Pt presented after episode at grocery store. Witness described shaking of arms and legs, pt remained standing, no fall.

In ED, episode observed:  
"asynchronous limb movements, stops with verbal redirection."

Labs normal.  
No imaging repeated.

Assessment:  
Non-epileptic event vs atypical seizure.

Plan:  
Neurology follow-up  
Consider video EEG if persists

Discharge Condition:  
Stable

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#### MISC DOCUMENT (PHONE MESSAGE LOG)

02/10/2026

Pt called stating symptoms worsening, now “tremor all day.”

02/12/2026

Spouse called, concerned about “possible Parkinson’s,” requesting urgent referral.

02/20/2026

Pt reports episodes triggered by stress, but also “random.”

02/25/2026

Staff note: patient very anxious, requests second opinion at Hopkins.

Fax ends abruptly

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